Thyrocare

Tests you can trust

**Name** : Ranjan Kumar Mohanty(44Y/M)

**Date** : 13 Aug 2025

**Test Asked** : Package 3 At Home

**Report Status:** Complete Report


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**98% Reports** released within **06 Hours** of sample reaching the lab+

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ISO 9001: 2015 - From 2015

CAP From 2007~

**PROCESSED AT :**
Thyrocare,
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Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

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**NAME** : RANJAN KUMAR MOHANTY(44Y/M)
**REF. BY** : SELF
**TEST ASKED** : PACKAGE 3 AT HOME

**Home Collection**
office chola ms general insurance company lmt,
naya bazar , badrak, near rajghat bridge

## Report Availability Summary

**Note:** Please refer to the table below for status of your tests.

**13** Ready **0** Ready with Cancellation **0** Processing **0** Cancelled in Lab

| TEST DETAILS | REPORT STATUS |
|---|---|
| **PACKAGE 3 AT HOME** | Ready |
| ERYTHROCYTE SEDIMENTATION RATE (ESR) | Ready |
| HEMOGRAM - 6 PART (DIFF) | Ready |
| CREATININE - SERUM | Ready |
| HbA1c | Ready |
| URIC ACID | Ready |
| SERUM ELECTROLYTES | Ready |
| LIPID PROFILE | Ready |
| T3-T4-USTSH | Ready |
| FASTING BLOOD SUGAR(GLUCOSE) | Ready |
| ROUTINE URINE ANALYSIS | Ready |
| ALANINE TRANSAMINASE (SGPT) | Ready |
| 25-OH VITAMIN D (TOTAL) | Ready |
| ASPARTATE AMINOTRANSFERASE (SGOT ) | Ready |

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MC-5500

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## Summary Report

### Tests outside reference range

| TEST NAME | OBSERVED VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|
| **COMPLETE HEMOGRAM** | | | |
| MEAN PLATELET VOLUME(MPV) | 12.1 | fL | 6.5-12 |
| MONOCYTES - ABSOLUTE COUNT | 0.17 | X $10^3$ / µL | 0.2 - 1.0 |
| PLATELET DISTRIBUTION WIDTH(PDW) | 16 | fL | 9.6-15.2 |
| TOTAL RBC | 5.67 | X 10^6/µL | 4.5-5.5 |
| **DIABETES** | | | |
| AVERAGE BLOOD GLUCOSE (ABG) | 126 | mg/dL | 90-120 |
| HbA1c | 6 | % | < 5.7 |
| **LIPID** | | | |
| HDL / LDL RATIO | 0.29 | Ratio | > 0.40 |
| HDL CHOLESTEROL - DIRECT | 30 | mg/dL | 40-60 |
| LDL CHOLESTEROL - DIRECT | 104 | mg/dL | < 100 |
| NON-HDL CHOLESTEROL | 160.8 | mg/dL | < 160 |
| TC/ HDL CHOLESTEROL RATIO | 6.4 | Ratio | 3 - 5 |
| TRIG / HDL RATIO | 13.94 | Ratio | < 3.12 |
| TRIGLYCERIDES | 418 | mg/dL | < 150 |
| VLDL CHOLESTEROL | 83.62 | mg/dL | 5 - 40 |
| **LIVER** | | | |
| ALANINE TRANSAMINASE (SGPT) | 48.8 | U/L | < 45 |
| **VITAMIN** | | | |
| 25-OH VITAMIN D (TOTAL) | 15.9 | ng/mL | 30-100 |

**Disclaimer**: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **HbA1c** | **H.P.L.C** | **6** | **%** |

**Bio. Ref. Interval. :**

| As per ADA Guidelines | Guidance For Known Diabetics |
|---|---|
| Below 5.7% : Normal | Below 6.5% : Good Control |
| 5.7% - 6.4% : Prediabetic | 6.5% - 7% : Fair Control |
| >=6.5% : Diabetic | 7.0% - 8% : Unsatisfactory Control |
| | >8% : Poor Control |

**Method :** Fully Automated H.P.L.C method

| | | | |
|---|---|---|---|
| **AVERAGE BLOOD GLUCOSE (ABG)** | **CALCULATED** | **126** | **mg/dL** |

**Bio. Ref. Interval. :**

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

**Method :** Derived from HBA1c values

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 13 Aug 2025 09:25
**Sample Received on (SRT)** : 13 Aug 2025 18:37
**Report Released on (RRT)** : 13 Aug 2025 21:00
**Sample Type** : EDTA Whole Blood
**Labcode** : 1308043756/DF543
**Barcode** : EM877879

Dr Anubhav A Sahu MD (Path)

Dr Satotsna Patra MD (Path



**Scan QR code to verify authenticity of reported results; active for 30 days from release time.**

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| ERYTHROCYTE SEDIMENTATION RATE (ESR) | MODIFIED WESTERGREN | 10 | mm / hr |

**Bio. Ref. Interval. :-**

Male : 0-15 Female : 0-20

Clinical Significance:
- An erythrocyte sedimentation rate (ESR) is a blood test that can rise if you have inflammation in your body. Its also used as a marker to monitor prognosis of an existing inflammatory/infective condition.
- Inflammation is your immune systems response to injury, infection, and many types of conditions, including immune system disorders, certain cancers and blood disorders.
- A high ESR test result may be from a condition that causes inflammation, such as: Arteritis, Arthritis, Systemic vasculitis, Polymyalgia rheumatica, Inflammatory bowel disease, Kidney disease, Infections like Tuberculosis etc, Rheumatoid arthritis and other autoimmune diseases, Heart disease, Certain cancers and many other Conditions.
- A low ESR test result may be caused by conditions such as: A blood disorder, such as: Polycythemia, Sickle cell disease (SCD), Leukocytosis, Heart failure, Certain kidney and liver problems etc.
- Certain physiological conditions also affect ESR results, these include : Pregnancy, menstrual cycle, ageing, obesity, drinking alcohol regularly, and exercise, Certain medicines and supplements also can affect ESR results.
- Hence Its always suggested to interpret ESR results in conjunction with Clinical History and other findings.

References :
https://medlineplus.gov/lab-tests/erythrocyte-sedimentation-rate-esr/

**Please correlate with clinical conditions.**
**Method:-** MODIFIED WESTERGREN

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| TEST NAME | METHODOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| **HEMOGLOBIN** | SLS-Hemoglobin Method | 15.4 | g/dL | 13.0-17.0 |
| Hematocrit (PCV) | CPH Detection | 48.9 | % | 40.0-50.0 |
| **Total RBC** | **HF & EI** | **5.67** | **X 10^6/µL** | **4.5-5.5** |
| Mean Corpuscular Volume (MCV) | Calculated | 86.2 | fL | 83.0-101.0 |
| Mean Corpuscular Hemoglobin (MCH) | Calculated | 27.2 | pq | 27.0-32.0 |
| Mean Corp.Hemo. Conc (MCHC) | Calculated | 31.5 | g/dL | 31.5-34.5 |
| Red Cell Distribution Width - SD (RDW-SD) | Calculated | 39.8 | fL | 39-46 |
| Red Cell Distribution Width (RDW - CV) | Calculated | 12.7 | % | 11.6-14 |
| RED CELL DISTRIBUTION WIDTH INDEX (RDWI) | Calculated | 193.1 | - | *Refer Note below |
| MENTZER INDEX | Calculated | 15.2 | - | *Refer Note below |
| **TOTAL LEUCOCYTE COUNT (WBC)** | HF & FC | 6.96 | X $10^3$ / µL | 4.0 - 10.0 |
| **DIFFERENTIAL LEUCOCYTE COUNT** | | | | |
| Neutrophils Percentage | Flow Cytometry | 64 | % | 40-80 |
| Lymphocytes Percentage | Flow Cytometry | 31 | % | 20-40 |
| Monocytes Percentage | Flow Cytometry | 2.4 | % | 2-10 |
| Eosinophils Percentage | Flow Cytometry | 2 | % | 1-6 |
| Basophils Percentage | Flow Cytometry | 0.3 | % | 0-2 |
| Immature Granulocyte Percentage (IG%) | Flow Cytometry | 0.3 | % | 0-0.5 |
| Nucleated Red Blood Cells % | Flow Cytometry | 0.01 | % | 0.0-5.0 |
| **ABSOLUTE LEUCOCYTE COUNT** | | | | |
| Neutrophils - Absolute Count | Calculated | 4.45 | X $10^3$ / µL | 2.0-7.0 |
| Lymphocytes - Absolute Count | Calculated | 2.16 | X $10^3$ / µL | 1.0-3.0 |
| **Monocytes - Absolute Count** | **Calculated** | **0.17** | **X $10^3$ / µL** | **0.2 - 1.0** |
| Basophils - Absolute Count | Calculated | 0.02 | X $10^3$ / µL | 0.02 - 0.1 |
| Eosinophils - Absolute Count | Calculated | 0.14 | X $10^3$ / µL | 0.02 - 0.5 |
| Immature Granulocytes (IG) | Calculated | 0.02 | X $10^3$ / µL | 0-0.3 |
| Nucleated Red Blood Cells | Calculated | 0.01 | X $10^3$ / µL | 0.0-0.5 |
| **PLATELET COUNT** | HF & EI | 195 | X $10^3$ / µL | 150-410 |
| **Mean Platelet Volume (MPV)** | **Calculated** | **12.1** | **fL** | **6.5-12** |
| **Platelet Distribution Width (PDW)** | **Calculated** | **16** | **fL** | **9.6-15.2** |
| Platelet to Large Cell Ratio (PLCR) | Calculated | 42.3 | % | 19.7-42.4 |
| Plateletcrit (PCT) | Calculated | 0.24 | % | 0.19-0.39 |

**Remarks :** Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

**Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)**

**(Reference** : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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**Sample Type** : EDTA Whole Blood
**Labcode** : 1308043756/DF543
**Barcode** : EM877879

Dr Anubhav A Sahu MD (Path)

Dr Satotsna Patra MD (Path)

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**TEST ASKED** : PACKAGE 3 AT HOME

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| FASTING BLOOD SUGAR(GLUCOSE) | PHOTOMETRY | 79 | mg/dL |

**Bio. Ref. Interval. :-**

| As per ADA Guideline: Fasting Plasma Glucose (FPG) | |
|---|---|
| **Normal** | 70 to 100 mg/dl |
| **Prediabetes** | 100 mg/dl to 125 mg/dl |
| **Diabetes** | 126 mg/dl or higher |

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

**Please correlate with clinical conditions.**
**Method:-** GOD-PAP METHOD

**Sample Collected on (SCT)** : 13 Aug 2025 09:25
**Sample Received on (SRT)** : 13 Aug 2025 18:27
**Report Released on (RRT)** : 13 Aug 2025 19:44
**Sample Type** : FLUORIDE PLASMA
**Labcode** : 1308103655/DF543
**Barcode** : EL521824

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| TEST NAME | METHODOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| **Complete Urinogram** | | | | |
| **Physical Examination** | | | | |
| VOLUME | Visual Determination | 4 | mL | - |
| COLOUR | Visual Determination | PALE YELLOW | - | Pale Yellow |
| APPEARANCE | Visual Determination | CLEAR | - | Clear |
| SPECIFIC GRAVITY | pKa change | 1.015 | - | 1.003-1.030 |
| PH | pH indicator | 5.5 | - | 5-8 |
| **Chemical Examination** | | | | |
| URINARY PROTEIN | PEI | ABSENT | mg/dL | Absent |
| URINARY GLUCOSE | GOD-POD | ABSENT | mg/dL | Absent |
| URINE KETONE | Nitroprusside | ABSENT | mg/dL | Absent |
| URINARY BILIRUBIN | Diazo coupling | ABSENT | mg/dL | Absent |
| UROBILINOGEN | Diazo coupling | Normal | mg/dL | <=0.2 |
| URINE BLOOD | Peroxidase reaction | ABSENT | - | Absent |
| NITRITE | Diazo coupling | ABSENT | - | Absent |
| LEUCOCYTE ESTERASE | Esterase reaction | ABSENT | - | Absent |
| **Microscopic Examination** | | | | |
| URINARY LEUCOCYTES (PUS CELLS) | Microscopy | 2 | cells/HPF | 0-5 |

**(Reference** : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

**Sample Collected on (SCT)** : 13 Aug 2025 09:25
**Sample Received on (SRT)** : 13 Aug 2025 18:53
**Report Released on (RRT)** : 13 Aug 2025 21:30
**Sample Type** : URINE
**Labcode** : 1308104611/DF543
**Barcode** : EF385688

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **25-OH VITAMIN D (TOTAL)** | **E.C.L.I.A** | **15.9** | **ng/mL** |

**Bio. Ref. Interval. :-**

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

**Please correlate with clinical conditions.**
**Method:-** Fully Automated Electrochemiluminescence Compititive Immunoassay

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**Sample Type** : SERUM
**Labcode** : 1308103854/DF543
**Barcode** : EJ259574

Dr Anubhav A Sahu MD (Path)
Dr Satotsna Patra MD (Path)

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| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| TOTAL CHOLESTEROL | PHOTOMETRY | 191 | mg/dL | < 200 |
| **HDL CHOLESTEROL - DIRECT** | **PHOTOMETRY** | **30** | **mg/dL** | **40-60** |
| **LDL CHOLESTEROL - DIRECT** | **PHOTOMETRY** | **104** | **mg/dL** | **< 100** |
| **TRIGLYCERIDES** | **PHOTOMETRY** | **418** | **mg/dL** | **< 150** |
| **TC/ HDL CHOLESTEROL RATIO** | **CALCULATED** | **6.4** | **Ratio** | **3 - 5** |
| **TRIG / HDL RATIO** | **CALCULATED** | **13.94** | **Ratio** | **< 3.12** |
| LDL / HDL RATIO | CALCULATED | 3.5 | Ratio | 1.5-3.5 |
| **HDL / LDL RATIO** | **CALCULATED** | **0.29** | **Ratio** | **> 0.40** |
| **NON-HDL CHOLESTEROL** | **CALCULATED** | **160.8** | **mg/dL** | **< 160** |
| **VLDL CHOLESTEROL** | **CALCULATED** | **83.62** | **mg/dL** | **5 - 40** |

**Please correlate with clinical conditions.**

**Method :**

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

| TOTAL CHOLESTEROL | (mg/dl) | HDL | (mg/dl) | LDL | (mg/dl) | TRIGLYCERIDES | (mg/dl) |
|---|---|---|---|---|---|---|---|
| DESIRABLE | <200 | LOW | <40 | OPTIMAL | <100 | NORMAL | <150 |
| BORDERLINE HIGH | 200-239 | HIGH | >60 | NEAR OPTIMAL | 100-129 | BORDERLINE HIGH | 150-199 |
| HIGH | >240 | | | BORDERLINE HIGH | 130-159 | HIGH | 200-499 |
| | | | | HIGH | 160-189 | VERY HIGH | >500 |
| | | | | VERY HIGH | >190 | | |

**Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.**

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| SODIUM | I.S.E - INDIRECT | 137.5 | mmol/L |

**Bio. Ref. Interval. :**
Adults: 136-145 mmol/l
**Method :** ION SELECTIVE ELECTRODE - INDIRECT

| | | | |
|---|---|---|---|
| POTASSIUM | I.S.E - INDIRECT | 4.26 | mmol/L |

**Bio. Ref. Interval. :**
ADULTS: 3.5-5.1 MMOL/L

Clinical Significance :
An abnormal increase in potassium (hyperkalemia)can profoundly affect the nervous system and increase the chance of irregular heartbeats (arrhythmias), which ,when extreme ,can be fatal. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Potassium in a given specimen may vary due to differences in assay methods, calibration and reagent specificity.
**Method :** ION SELECTIVE ELECTRODE - INDIRECT

| | | | |
|---|---|---|---|
| CHLORIDE | I.S.E - INDIRECT | 102.5 | mmol/L |

**Bio. Ref. Interval. :**
ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).
**Method :** ION SELECTIVE ELECTRODE - INDIRECT

**Please correlate with clinical conditions.**

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| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| SGOT / SGPT RATIO | CALCULATED | 0.67 | Ratio | < 2 |
| ASPARTATE AMINOTRANSFERASE (SGOT ) | PHOTOMETRY | 32.8 | U/L | < 35 |
| **ALANINE TRANSAMINASE (SGPT)** | **PHOTOMETRY** | **48.8** | **U/L** | **< 45** |

**Please correlate with clinical conditions.**

**Method :**

OT/PT - Derived from SGOT and SGPT values.
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| CREATININE - SERUM | PHOTOMETRY | 0.97 | mg/dL |

**Bio. Ref. Interval. :-**

Male : 0.72 -1.18 mg/dL
Female: 0.55 - 1.02 mg/dL

Clinical Significance :
The significance of a single creatinine value must be interpreted in light of the patients muscle mass. A patient with a greater muscle mass will have a higher creatinine concentration. The trend of serum creatinine concentrations over time is more important than absolute creatinine concentration. Serum creatinine concentrations may increase when an ACE inhibitor (ACEI) is taken. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic.

**Please correlate with clinical conditions.**
**Method:-** Creatinine Enzymatic Method

**Sample Collected on (SCT)** : 13 Aug 2025 09:25
**Sample Received on (SRT)** : 13 Aug 2025 18:32
**Report Released on (RRT)** : 13 Aug 2025 20:17
**Sample Type** : SERUM
**Labcode** : 1308103854/DF543
**Barcode** : EJ259574

Dr Anubhav A Sahu MD (Path)
Dr Satotsna Patra MD (Path)

**PROCESSED AT :**

Thyrocare,
M. J. Plaza, Mahaveer Nagar,
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Bhubaneswar – 751 006

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**NAME** : RANJAN KUMAR MOHANTY(44Y/M)
**REF. BY** : SELF
**TEST ASKED** : PACKAGE 3 AT HOME

**Home Collection**
office chola ms general insurance company lmt, naya bazar , badrak, near rajghat bridge

| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| TOTAL TRIIODOTHYRONINE (T3) | E.C.L.I.A | 128 | ng/dL | 80-200 |
| TOTAL THYROXINE (T4) | E.C.L.I.A | 9.21 | µg/dL | 4.8-12.7 |
| TSH - ULTRASENSITIVE | E.C.L.I.A | 3.11 | µIU/mL | 0.54-5.30 |

**Comments :** ***

**The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.**

**Method :**

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

**Disclaimer :**Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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**TEST ASKED** : PACKAGE 3 AT HOME

**Home Collection**
office chola ms general insurance company lmt, naya bazar , badrak, near rajghat bridge

| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| URIC ACID | PHOTOMETRY | 5.5 | mg/dL | 4.2 - 7.3 |

**Please correlate with clinical conditions.**

**Method :**

URIC - Uricase / Peroxidase Method

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| EST. GLOMERULAR FILTRATION RATE (eGFR) | CALCULATED | 99 | mL/min/1.73 m2 |

**Bio. Ref. Interval. :-**

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

**Please correlate with clinical conditions.**
**Method:-** 2021 CKD EPI Creatinine Equation

**~~ End of report ~~**

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**Scan QR code to verify authenticity of reported results; active for 30 days from release time.**

## CONDITIONS OF REPORTING

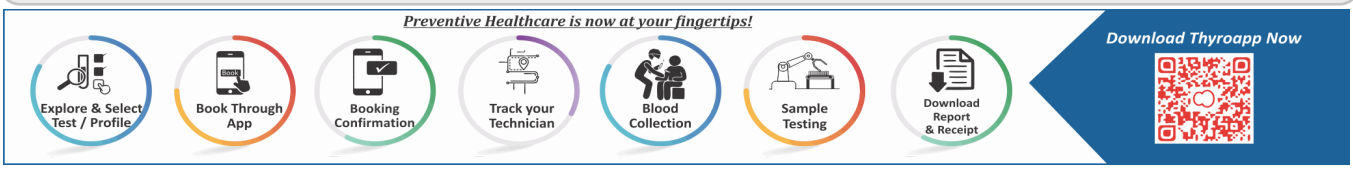
- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

## EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

## SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**

+ *T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),*
* *As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited*